

Ezede`

Oral Solution 1mg/ml

COMPOSITION

Each 5ml contains Loratadine 5mg.
Sodium Benzoate 0.1%w/v as preservative.

PHARMACODYNAMICS

Loratadine is a long-acting, non-sedating H₁-antagonist antihistamine with no significant antimuscarinic activity.

PHARMACOKINETICS

Loratadine is rapidly absorbed from the gastrointestinal tract after oral doses, peak plasma concentrations being attained in about 1 hour. Bioavailability is increased and time to peak plasma concentrations is delayed when taken with food.

Loratadine undergoes extensive metabolism. The major metabolite, desloratadine has potent antihistaminic activity. Reported mean elimination half-lives for loratadine and desloratadine are 8.4 and 28 hours respectively.

Loratadine is about 98% bound to plasma proteins and desloratadine is less extensively bound. Most of the dose is excreted equally in the urine and faeces, mainly in the form of metabolites.

INDICATIONS

Loratadine is indicated for the relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge (rhinorrhea) and itching, as well as ocular itching and burning. Nasal and ocular signs and symptoms are relieved rapidly after oral administration.

Loratadine is also indicated for relief of symptoms and signs of chronic urticaria and other allergic dermatologic disorders.

DOSAGE AND ADMINISTRATION

For oral use only.

Given as a single daily dose, with or without food.

- Adults and Children 12 years of age and over: Two 5 ml spoonfuls (10 mg) once daily.
- Children 6 to 12 years of age:
Body Weight >30 kg: Two 5 ml spoonfuls (10 mg) syrup once daily.

- Body Weight <30 kg: One 5 ml spoonful (5 mg) syrup once daily.
- Children 2 to 6 years of age:
Body Weight >30 kg: Two 5 ml spoonfuls (10 mg) syrup once daily.
Body Weight <30 kg: One 5 ml spoonful (5 mg) syrup once daily.

Safety and efficacy of Loratadine has not been established in children younger than 2 years of age.

Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5 mg or 5 ml once daily, or 10 mg or 10 ml every other day is recommended.

CONTRAINDICATIONS

Contraindicated in patients with known hypersensitivity to loratadine or to any of the excipients.

WARNINGS AND PRECAUTIONS

Should be administered with caution in patients with severe liver impairment.

The administration of this product should be discontinued for at least 48 hours before skin tests since antihistamines may prevent or reduce otherwise positive reactions to dermal reactivity index.

Loratadine has no or negligible influence on the ability to drive and use machines. Rarely, some people may experience drowsiness, which may affect the ability to drive or use machines. Avoid taking alcoholic drink or other drugs that may cause drowsiness.

DRUG INTERACTIONS

Loratadine is metabolized by cytochrome P450 isoenzymes CYP3A4 and CYP2D6. Therefore, use with other drugs that inhibit or are metabolized by these hepatic enzymes may result in changes in plasma concentrations of either drug, or possibly adverse effects. Drugs known to inhibit one or other of these enzymes include cimetidine, erythromycin, ketoconazole, quinidine, fluconazole and fluoxetine.

PREGNANCY AND LACTATION

Pregnancy

The use of loratadine during pregnancy has not been established. As a precautionary measure, it is preferable to avoid the use of loratadine during pregnancy.

Lactation

Loratadine is excreted in breast milk. Therefore, the use of loratadine is not recommended in breastfeeding.

SIDE EFFECTS

Frequency	Side effect
Common	Somnolence, headache, increased appetite and insomnia
Very rare	Hypersensitivity reaction (including angioedema and anaphylaxis), dizziness, convulsion, tachycardia, palpitation, nausea, dry mouth, gastritis, abnormal hepatic function, rash, alopecia and fatigue

SYMPTOMS AND TREATMENT FOR OVERDOSAGE

Overdosage with loratadine increased the occurrence of anticholinergic symptoms. Somnolence, tachycardia and headache have been reported with overdoses.

In the event of overdose, general symptomatic and supportive measures are to be instituted and maintained for as long as necessary. Administration of activated charcoal as a slurry with water may be attempted. Gastric lavage may be considered. Loratadine is not removed by hemodialysis and it is not known if loratadine is removed by peritoneal dialysis. Medical monitoring of the patient is to be continued after emergency treatment.

STORAGE CONDITION

Store below 30°C.

SHELF LIFE

The expiry date is indicated on the packaging.

PRODUCT DESCRIPTION, DOSAGE FORM AND PACKAGING AVAILABLE

Clear, pink colour oral solution with vanilla blackcurrant flavour.

Available in pack size of 60ml, 90ml and 120ml.

For further information, please consult your physician or pharmacist.

Revision Date: 21-Sep-2021

Manufacturer and Product Registration Holder

Xepa-Soul Pattinson (Malaysia) Sdn Bhd

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